

Patient ID: GarrettPeriod: April 1 to April 18, 2026Report Date: April 19, 2026Coverage: 365/430h (85%)

Patient's monitoring period was less than 30 days; this report covers all available data. Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

Elevated Breathing > 24 brpmElevated HR > 95 bpmHigh HR > 100 bpm

Last 24 Hours

Elevated Breathing			
11 PM to 4 AM			
Vital Stat	Avg	Min	Max
Heart Rate	61 bpm	50 bpm	72 bpm
Breathing	23 brpm	14 brpm	47 brpm

Last 24 hours: three elevated breathing episodes in the evening.

Episodic Events (last 24h)

Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
11 PM to 4 AM	7h	3	Elevated Breathing	25	15	47	Check SpO2 and lung sounds; assess for fluid overload or early infection

Patient clinical status over 18 days from Apr 01 to Apr 18

■ HR ■ Breathing

Elevated Breathing							High Breathing
Apr 13 to Apr 18 (6d)							Apr 14 to Apr 14 (1d)

17 episodic events Detected, spanning 38h total. Conditions: Elevated Breathing, High Breathing. Priority grade: High

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
17	38h	Elevated Breathing	<1	85%

Major Findings: Sustained elevated breathing (avg 26, peak 57)

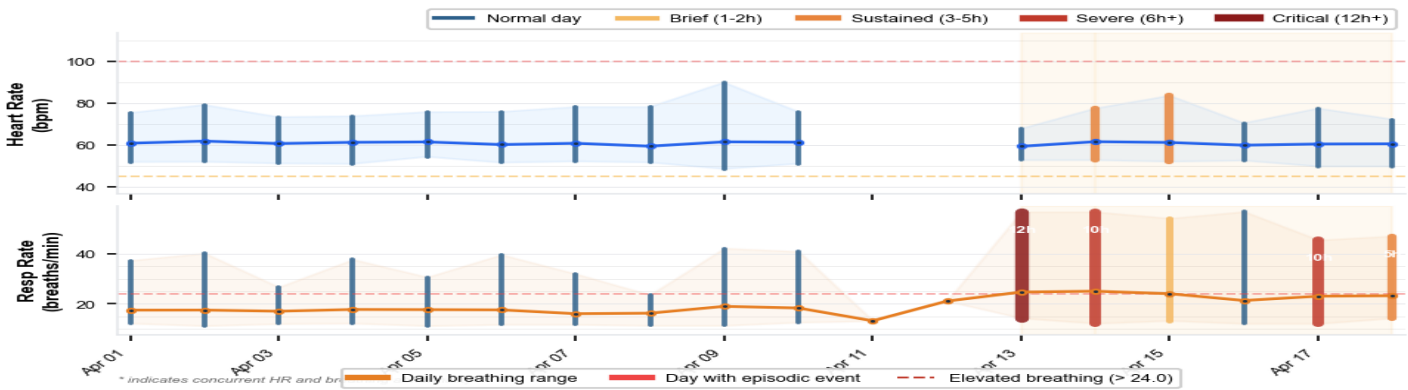
Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Apr 14	11 PM to 2 AM	3h	1	High Breathing	33	23	57	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Apr 13	6 PM to 7 PM	35h	16	Elevated Breathing	26	14	57	Check SpO2 and lung sounds; assess for fluid overload or early infection

Suggested Clinical Review Actions

- Elevated Breathing (avg 26 brpm, peak 57 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- High Breathing (avg 33 brpm, peak 57 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Overall trajectory shows 2 unstable phases with 9 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

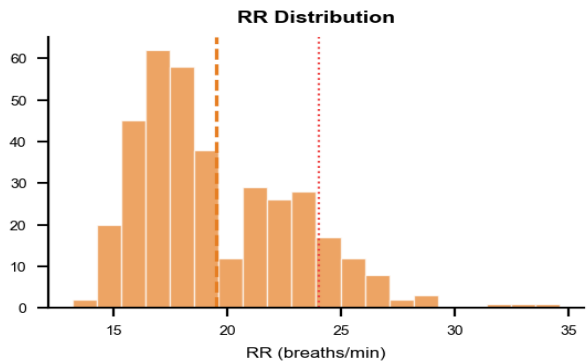
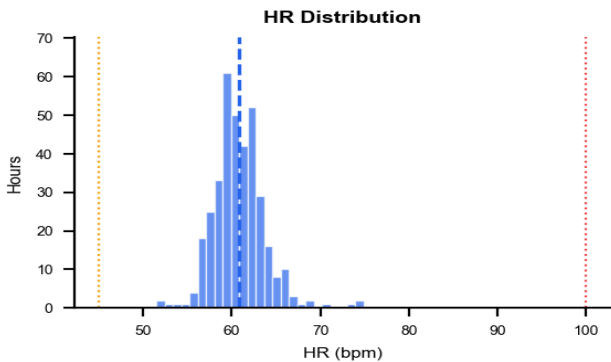
Clinical Pattern Observations:

- 1. Sustained finding: 10h continuous Elevated Breathing beginning Apr 13.
- 2. Breathing variability: RR spread 10 brpm (15 to 25).
- 3. Clustered pattern: Episodic events on 5 of 18 days (27%).

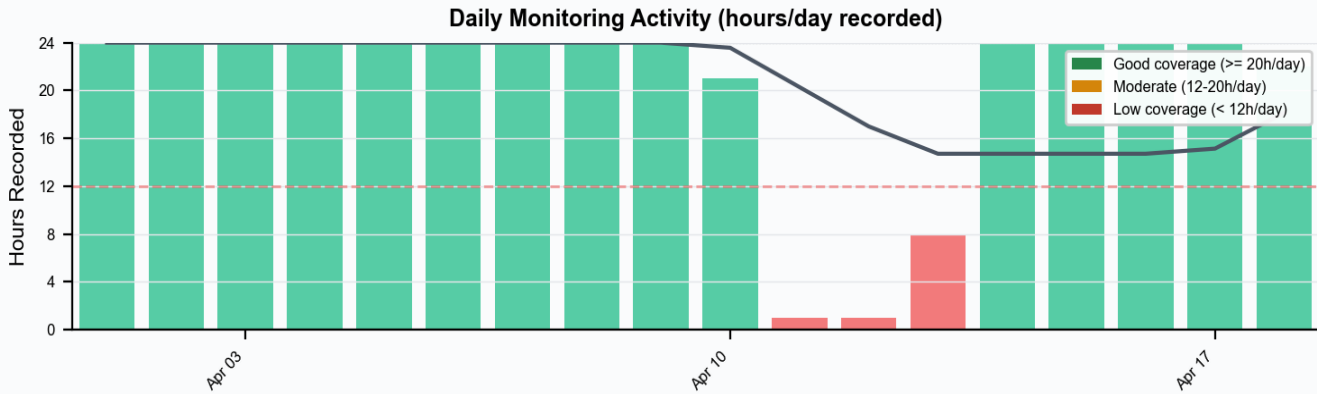


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.